

PRODUCT FEATURE

PROMs and Questionnaires

What is a PROM?

A Patient Reported Outcome Measure is any report of a person's health that comes directly from them, without a clinician or anyone else interpreting the answer on their behalf [1]. It is captured through a standardised set of questions, asked the same way every time, so answers can be compared between people and tracked over time.

That matters because a screening measures what a person can do, not what they experience. Whether someone has already fallen, feels unsteady on their feet, or has begun avoiding activities because they worry about falling is invisible to a sensor and reachable only by asking. These are precisely the questions international falls guidance puts first [2][3].

One questionnaire engine, several uses

What sits behind PROMs on the Able Platform is a configurable questionnaire engine. PROMs are its primary use, but the same engine captures the other structured input a screening needs:

Use	Scoring	Purpose
PROM	Usually scored	A standardised instrument completed by, or on behalf of, the individual, either on its own or as a step within a screening
Screening skip reason	Scored	Records why a test was not performed, scored so that skips can contribute to the falls risk calculation
Protocol deviation	Unscored	Records how a test departed from protocol, giving the caregiver context when interpreting that result

Every questionnaire is either scored or unscored. A scored questionnaire attaches a value to each answer, aggregates the values given, and returns an at-risk outcome when the total reaches the questionnaire's threshold. An unscored questionnaire validates and stores the answers without producing a score or an outcome, which is what you want when the answers are contextual rather than a measurement.

Scoring always runs on our servers rather than on the device, so a result is computed the same way every time and cannot be altered by a client. Submission is all or nothing, so there are no partial records to interpret, and free-text answers never contribute to a score.

Our first PROM: STEADI 3 Key Questions

The 3 Key Questions are three items drawn from the CDC's 12-item Stay Independent questionnaire, used worldwide as a first-pass falls screen and endorsed by the CDC as a standalone instrument [2][4]. The threshold is 1, so a single Yes marks the person at risk, matching the CDC STEADI algorithm, which has been validated against 7,392 adults aged 65 and over [5]. It takes about 60 seconds.

A scored questionnaire in practice

Question	Answer	Value
Have you fallen in the past year?	No	0
Do you feel unsteady when standing or walking?	Yes	1
Do you worry about falling?	Yes	1
Total		2
PROM risk, against a threshold of 1		At risk

This person has not fallen, so a falls-history question alone would have missed her. Worry about falling predicts future falls rather than only following them [6], and no sensor can pick that up.

An unscored questionnaire in practice

An invented example, showing what the engine can do rather than a questionnaire we ship.

Question	Type	Answer
How did today's session feel?	Single select	Harder than usual
What made it harder? Asked only when the previous answer is "Harder than usual"	Multiple choice with other	Tiredness, and Other: "Slept badly"
Anything else your clinician should know?	Free text	"Right knee was sore afterwards"

No score and no outcome are produced. The answers are stored and presented with the session, so the clinician has the context when they review the results.

How PROM risk relates to the other signals

PROM risk is stored and exposed as its own signal, separate from the measurement-based falls risk flag and the Able Score. Keeping them distinct matters: a combined number would hide which signal fired, and the clinical response to a mobility problem is not the response to a person who is frightened of falling. Where you do want them to interact, PROM responses and skip reasons captured during a screening can be configured to contribute to the total falls risk calculation, with the PROM outcome still visible in its own right.

Building your own questionnaires

The engine is not falls specific. A questionnaire is an ordered set of questions using any of these question types:

Type	Behaviour
Single select	The person picks one of the offered options
Single select with other	As above, plus a free-text "Other" answer
Multiple choice	The person picks any number of options; when scored, the selected values are aggregated
Multiple choice with other	As above, plus a free-text "Other" answer
Free text	An open written answer, up to 4,000 characters

Any question can be made dependent on an earlier answer, appearing only when that answer was given, so a questionnaire can branch rather than grow long. Once a questionnaire has been used its definition is locked and responses are stored against the exact version that produced them, so revising one means publishing a new version rather than editing history, and historical records stay interpretable.

Between them these types cover most standard instruments: symptom and severity scales including the PHQ-9, Likert-scale batteries, patient satisfaction and technology acceptance surveys, research instruments such as the Intrinsic Motivation Inventory, and any bespoke set of questions your service needs. Care homes, community services and research studies can each run the questionnaires their setting requires in tandem with the sensor-based measurements.

Availability

In Able Assess, a PROM can be standalone or completed as an ordered step within a screening. Outcomes are returned through the platform's API, both per assessment and as an independent risk status on the screening, with results soon to be available through the Able Care web portal, in pdf reports

and through third-party integrations. Which questionnaires are available is set centrally, without an app update, and applies globally, per organisation or per facility: we control which are enabled, what each is called on screen, whether its result is shown, where it sits in a screening, whether it can be skipped, and whether skipping requires a recorded reason.

References

1. US Food and Drug Administration. Guidance for Industry. Patient-Reported Outcome Measures: Use in Medical Product Development to Support Labeling Claims. 2009. Source of the PRO definition.
2. Centers for Disease Control and Prevention. STEADI Algorithm for Fall Risk Screening, Assessment and Intervention. <https://www.cdc.gov/steady/media/pdfs/STEADI-Algorithm-508.pdf>
3. Montero-Odasso M, et al. World guidelines for falls prevention and management for older adults: a global initiative. *Age and Ageing*. 2022;51(9):afac205. doi:10.1093/ageing/afac205
4. Stevens JA, Phelan EA. Development of STEADI: a fall prevention resource for health care providers. *Health Promotion Practice*. 2013;14(5):706-714. doi:10.1177/1524839912463576
5. Lohman MC, Crow RS, DiMilia PR, Nicklett EJ, Bruce ML, Batsis JA. Operationalisation and validation of the Stopping Elderly Accidents, Deaths, and Injuries (STEADI) fall risk algorithm in a nationally representative sample. *Journal of Epidemiology and Community Health*. 2017;71(12):1191-1197.
6. Friedman SM, Munoz B, West SK, Rubin GS, Fried LP. Falls and fear of falling: which comes first? A longitudinal prediction model suggests strategies for primary and secondary prevention. *Journal of the American Geriatrics Society*. 2002;50(8):1329-1335.
7. Able Care. Able Assess: the multifactorial evidence behind falls risk screening. <https://able-care.co/blog/able-assess-multifactorial-evidence-falls-risk-screening>

Disclaimer: A PROM records what a person reports about themselves. Outcomes are computed against the questionnaire's published threshold and are intended to support clinical judgement, not to replace it, and they do not constitute a diagnosis or a prediction that a fall will occur.